

UHB Urgent Suspected Gynaecological Cancer Referral Form Excluding Endometrial / Uterine

This referral is made on the basis that the referring doctor considers that the patient has clinical indications of a new malignancy and mindful of NICE Guidelines

All USC referrals should be made through the NHS eReferral Service (eRS)

GYNAECOLOGY ADULT (Over 16 years)

It is essential that all appropriate boxes are completed in full to aid clinical triage and appropriate care of the patient. If they are not, referral may not be considered

PATIENT DEMOGRAPHICS		Date of referral:	
Surname: Surname		Address: Home Full Address (single line)	
First name: Given Name			
Gender: Male ☐ Female ☐			
NHS number: NHS Number			
Hospital Number: Hospital Number		Telephone	
Date of birth: Date of Birth		Day Time: Patient Mobile Telephone Evening: Patient Mobile Telephone	
Interpreter required:	Yes ☐ No ☐	Main Language:	

REASONABLE ADJUSTMENTS

Details: Single Code Entry: Requires reasonable adjustment for health and care access (Equality Act 2010)...

GP/REFERRER DETAILS	
Referrer: Free Text Prompt	Practice Name: Organisation Name
Usual GP: Usual GP Full Name	Practice Address: Organisation Full Address (single line)
GP Practice Code: Organisation National Practice Code	
Practice Telephone: Organisation Telephone Number	
Practice email: Organisation E-mail Address	

I have provided the patient with a 2 week wait information leaflet (leaflets available to order CRUK) Your urgent suspected cancer referral - text version (downloadable PDF) Publications (cancerresearchuk.org)	Yes ☐ No ☐
Is patient available in the next two weeks:	Yes ☐ No ☐

REASON FOR REFERRAL	TICK
Ovarian *Please note appropriate investigations	
1. Refer the woman urgently if physical examination identifies ascites and/or a pelvic or abdominal mass (which is not obviously uterine fibroids)	☐
2. Pelvic Ultrasound or CT/MRI imaging suggestive of Ovarian Cancer	☐
3. Elevated CA125 ≥ 35 in post-menopausal women with the below symptoms.	☐
4. Elevated CA125 ≥ 70 in pre-menopausal women with the below symptoms.	☐
(Elevated CA125 35-69 in pre-menopausal women with the below symptoms, do not use this form , instead request an URGENT Pelvic Ultrasound Scan - Ovarian Cancer Pathway on ICE)	
As Per NICE, the following symptoms should be referred if associated with elevated CA125: Persistent abdominal distention/bloating Feeling full (early satiety) and/or loss of appetite, pelvic or abdominal pain, increased urinary urgency and/or frequency, unexplained weight loss, fatigue, or changes in bowel habit.	CA125 Result : See Summary Sheet below. ☐ Requested

***To aid clinical triage please arrange prior to referral: U&E's required to facilitate CT scan with contrast if necessary. If you have a scan report, please attach it to this referral. It will stop patient from having another scan appointment.**

U&Es and eGFR Result (within last 3 months) **See Summary Sheet Below**

☐ Requested

Cervical *Postcoital bleeding and intermenstrual bleeding with a normal cervix is not an indication for 2WW

1. Patient has had unscheduled vaginal bleeding after subtotal hysterectomy
2. Appearance of the cervix on the speculum examination is consistent with cervical cancer: Suspicious lesion on cervix

☐
☐

Date of last cervical smear and result: **See Summary Sheet Below**

Any prev history of CIN yes ☐ or no ☐

Genital swabs yes ☐ or no ☐

Urine samples for STI yes ☐ or no ☐

Vulval/Vaginal

1. Unexplained lump or suspicious lesion (not prolapse)

☐

2. Bleeding due to ulceration

☐

Please detail any clinical findings and relevant clinical information in this section (including any recent investigations).

Please indicate whether the patient has had a hysterectomy

Yes ☐ No ☐

Is this a suspicion of recurrence of known Gynaecological Cancer

Yes ☐ No ☐

Weight (kg)

Single Code Entry: Body weight

Body Mass Index – within the last year

Single Code Entry: Body mass index

I have performed a gynaecological examination

Yes ☐ No ☐

Patient has been informed they may have to undergo a Transvaginal ultrasound scan

Yes ☐ No ☐

Patient leaflet regarding internal scan given

ASSESSMENT OF PATIENT'S SUITABILITY FOR DIAGNOSTIC TESTS

(To aid clinical triage & reduce delay in secondary care)

Are there any concerns about the patient's mental capacity at this particular moment in time Yes ☐ No ☐

I have assessed frailty / begun best interest discussion: Yes ☐ No ☐ N/A ☐

Please sign the declaration below for mental capacity and/or frail/WHO performance status of 3 and 4. For these patients, please consider a best interest discussion, taking into consideration the need for intrusive intimate examination, further investigations and suitability for treatment, if cancer is diagnosed.

PERFORMANCE STATUS – W.H.O GRADE (To aid clinical triage)

Able to carry out all normal activities without restriction.

0

☐

Restricted in physically strenuous activity. Able to walk and do light work.

1

☐

Able to walk and capable of all self-care. Unable to work. Up and about more than 50% of waking hours

2

☐

Capable of only limited self-care. Confined to bed or chair more than 50% of waking hours.

3

☐

Completely disabled. Cannot carry out any self-care. Totally confined to bed or chair.

4

☐

GP DECLARATION

I confirm that I have discussed where appropriate based on the patient's mental capacity the possibility that the diagnosis may be cancer and that they may need to undergo a range of tests

☐

'Please provide further details below if you are referring a patient with poor performance score or have concerns around patient capacity to consent to investigation'.

Please include Patient Information for medication, past medical history, repeat medications, allergies, recent investigations. (from Clinical System)

Problems

Allergies

Investigations

Medication

CA125: Single Code Entry: Serum CA 125 (cancer antigen 125) level...

Cervical Smear